

**PRIMETIL TABLET 5mg**

Description: Orange, round, flat top-scored tablet. Diameter=6.00mm.

Content

Prochlorperazine maleate 5mg per tablet

Indications

For prevention and control of nausea and vomiting; for treatment of Meniere's disease and other forms of vestibular disorder and for management of psychotic disorders especially schizophrenia and nonpsychotic emotional disturbances such as anxiety and tension.

Pharmacology

Prochlorperazine, a piperazine phenothiazine, acts by inhibiting the medullary chemoreceptor trigger zone in the control of mild or severe nausea and vomiting. It is thought to improve psychotic conditions by blocking postsynaptic mesolimbic dopaminergic receptors in the brain. As a tranquilizing agent, it causes indirect reduction of stimuli to the brainstem reticular system.

Prochlorperazine is well absorbed from the GI tract, subjectable to first pass metabolism. It is metabolised in the liver by hydroxylation and conjugation with glucuronic acid, N-oxidation, oxidation of the sulfur atom and dealkylation. Plasma half-life is reported to be only a few hours but elimination of the metabolites may be prolonged.

Prochlorperazine is extensively bound to plasma proteins, widely distributed in the body and its metabolites cross the placenta and excreted in milk. It is excreted in urine and bile. Rate of metabolism and excretion decreases in old age.

Pregnancy and lactation

Neonates exposed to antipsychotic drugs during the third trimester of pregnancy are at risk for extrapyramidal and/or withdrawal symptoms following delivery. There have been reports of agitation, hypertonia, hypotonia, tremor, somnolence, respiratory distress, and feeding disorder in these neonates. These complications have varied in severity; while in some cases symptoms have been self-limited, in other cases neonates have required intensive care unit support and prolonged hospitalisation.

Primetil tablet should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus.

Adverse effects

Incidence more common include parkinsonism or dystonic extrapyramidal effects and nasal congestion. Others may include drowsiness, dryness of the mouth, tachycardia, agitation, miosis and mydriasis, photosensitivity, allergic reactions, jaundice, haematological disorders and alteration of endocrine function.

Precautions

- Caution in alcoholic and diabetic patients with blood dyscrasias, liver, cardiovascular or respiratory disease and parkinsonism.
- It should be used with care in pregnant and lactating patients; geriatric and debilitated patients; and extreme caution in children.
- It is not recommended for children weighing less than 10kg.
- Caution when driving or doing jobs requiring alertness because of possible drowsiness.
- Check with physician before discontinuing medication; gradual dosage reduction may be necessary

Contraindications

Not recommended for patients with severe CNS depression, severe cardiovascular disease and comatose states.

Drug Interactions

- Concurrent use with narcotic analgesics, anticonvulsants and other CNS depressants may require dosage adjustment of these medications.
- Caution in patients receiving the following drug therapy:
levodopa, guanetidine, other extrapyramidal reaction causing medications and beta-adrenergic blocking agents.

Dosage and Administration

- Adults : Antiemetic/ antivertigo - Oral, 5 to 10mg 3 to 4 times a day; or as directed.
Antipsychotic - Oral, 5 to 35mg 3 to 4 times a day; the dose being increased gradually until the desired response is obtained, for which 50-150mg is usually required.
Non-psychotic emotional disturbance - Oral, 5 to 10mg 3 to 4 times a day.
- Children : Antiemetic - 2 to 5 years - Oral, 2.5mg 2 times a day; not exceeding 20mg daily.
- 6 to 12 years - Oral, 5mg 2 or 3 times a day; not exceeding 25mg daily.
Antipsychotic - 2 to 12 years - Oral, 2.5mg 2 or 3 times a day; or as directed.

Overdosage

Clinical features include hypotension, cardiac arrhythmias, convulsions, CNS depression, severe extrapyramidal reactions, tardive dyskinesia and low body temperature.

Treatment consists of gastric lavage; emetics should not be used. Symptomatic and supportive therapy is recommended.

Severe extrapyramidal reactions should be treated with a slow intravenous injection of benztropine mesylate or diphenhydramine.

Storage Conditions : Keep container tightly closed. Store in a cool, dry place below 30° C.
Protect from light. Keep away from children.

Shelf life : 3 years

Presentation / Packing : 100x10's/blister

Manufactured & : PRIME PHARMACEUTICAL SDN. BHD.

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